

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: TaylorPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 658/720h (91%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 30 days from Apr 01 to Apr 30

HR Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

| Vital      | Avg     | Min     | Max     |
|------------|---------|---------|---------|
| Heart Rate | 60 bpm  | 44 bpm  | 92 bpm  |
| Breathing  | 16 brpm | 10 brpm | 32 brpm |

Last 24 hours: one elevated breathing episode in the morning, otherwise within normal range.

Episodic Events (last 24h)

| Date   | Time Span    | Duration | Episodes | Condition          | Avg | Min | Max | Comment                                                                  |
|--------|--------------|----------|----------|--------------------|-----|-----|-----|--------------------------------------------------------------------------|
| Apr 30 | 6 AM to 7 AM | 1h       | 1        | Elevated Breathing | 26  | 15  | 32  | Check SpO2 and lung sounds; assess for fluid overload or early infection |

6 episodic events Detected, spanning 6h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Low

| Episodic Events | Total Hours | Highest Burden | Episodes/day | Coverage |
|-----------------|-------------|----------------|--------------|----------|
| 6               | 6h          | Low Heart Rate | <1           | 91%      |

Major Findings: Stable baseline

| Date   | Time Span      | Duration | Episodes | Condition          | Avg | Min | Max | Comment                                                                         |
|--------|----------------|----------|----------|--------------------|-----|-----|-----|---------------------------------------------------------------------------------|
| Apr 05 | 8 AM to 9 AM   | 1h       | 1        | Low Heart Rate     | 42  | 40  | 46  | Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic |
| Apr 16 | 3 AM to 4 AM   | 1h       | 1        | Low Heart Rate     | 44  | 40  | 51  | Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic |
| Apr 23 | 4 AM to 5 AM   | 1h       | 1        | Low Heart Rate     | 43  | 39  | 47  | Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic |
| Apr 26 | 10 PM to 11 PM | 1h       | 1        | Low Heart Rate     | 45  | 39  | 52  | Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic |
| Apr 12 | 9 AM to 10 AM  | 1h       | 1        | Elevated Breathing | 29  | 29  | 29  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |
| Apr 30 | 6 AM to 7 AM   | 1h       | 1        | Elevated Breathing | 26  | 15  | 32  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |

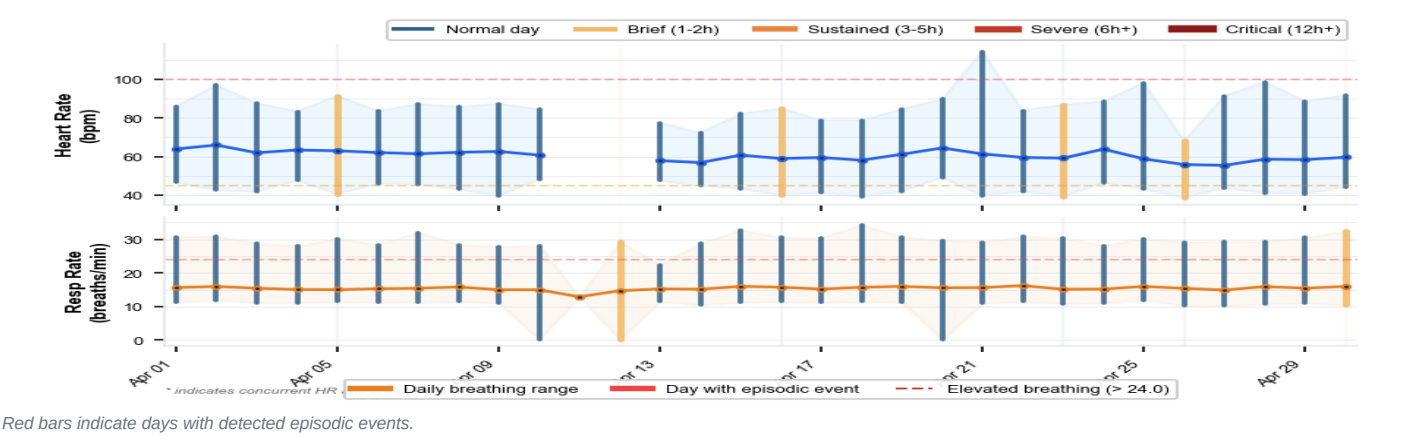
The P5 to P95 heart rate spread was 20 bpm (51 to 72), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

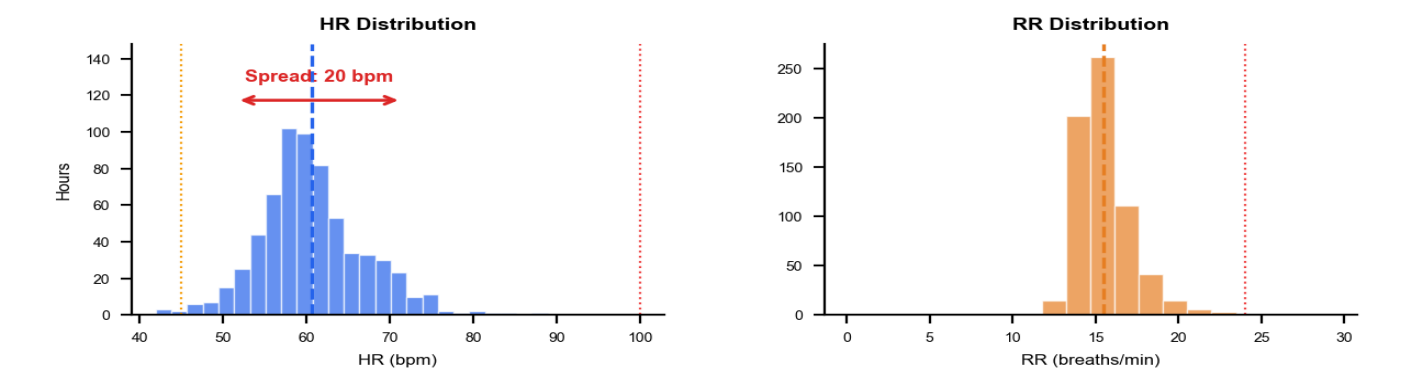
- Elevated Breathing (avg 29 brpm, peak 29 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 42 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 6 unstable phases with 20 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

Clinical Pattern Observations:

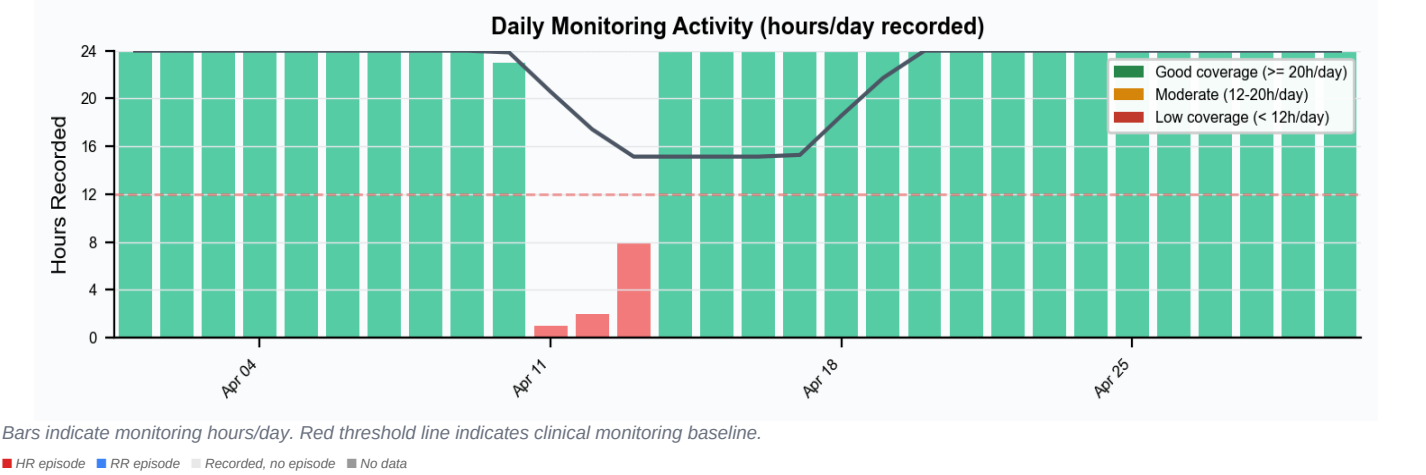
- 1. High variability: HR spread 20 bpm (51 to 71).
- 2. Clustered pattern: Episodic events on 6 of 30 days (20%).



Vital Sign Distribution



Monitoring Activity



Episodic event: threshold exceeded continuously for  $\geq 1$  hour. Episodes separated by  $\leq 1$  hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.